THE DECISION

Buy evidence before scale

A bounded joint diligence option. No partnership, licence or combined-platform approval is verified.

PROPOSED AUTHORIZATION

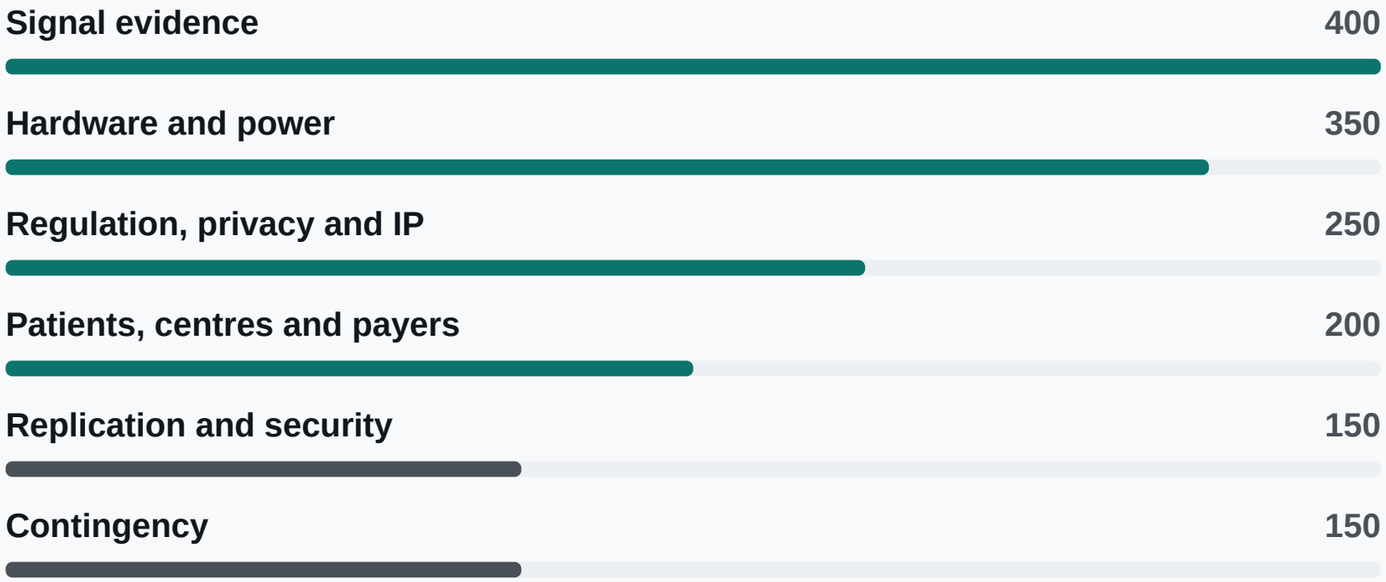
UP TO

$1.5m

90 days

Resolve rights, signal credibility, product scope and paid patient value.

SIX WORKSTREAMS / USD THOUSANDS



Base NPV: core -$95.5m | licensed -$2.1m

A + B / CONDITIONAL R&D

Hearing component + controlled assistant

C / HOLD HUMAN USE

Chronic tonsillar route remains unvalidated

RIGID ROUTE / NO-GO

Proposed tonsillar Utah-array approach

Basis: merged editorial controls; S1 decision/evidence review; S2 proposal pp. 1-5.

Later capital requires a new release decision. Sponsor/developer contribution shares remain to be negotiated.

PATIENT VALUE

Control the interaction. Prove the benefit.

Start with clinically eligible adult CI users and qualified interfaces. Benefit claims remain hypotheses.

01 HEAR
Compatible device / qualified gateway

→

02 ENABLE
Patient chooses assistant + context

→

03 REVIEW
Preview and edit suggested output

→

04 CONFIRM
Approve private output or an external action

→

05 OFF
End optional capture, processing and output

VALUE TO TEST

PATIENT

Less communication effort

Task success, corrections, unwanted output, fatigue and sustained preference.

CENTRE

Better support workflow

Troubleshooting time, calibration burden, actionable information and safe escalation.

OEM / PAYER

Benefit worth the total cost

Integration burden, durable outcomes, service cost and willingness to pay.

COMPARE AGAINST

Available CI / TICI
Hearing, burden, reliability, revision risk

Phone + native assistant
Task value beyond free or bundled tools

Noninvasive attention tools
Separate research; no therapy revenue

Sources: S2 proposal pp. 6, 21, 23-26; S2-SP008 (Bluetooth); S2-SCL006 (MED-EL); S2-AT012.

Auracast is compatible external audio distribution. A shared broadcast code is not personal medical authorization.

EVIDENCE + PATIENT CONTROL

Promising components. Unproven integration.

Measure the bridge between each component and the claimed patient outcome.

7 / 5
SENSORS / CADAVERIC EARS
UmboMic component evidence

SMALL
CORTICAL HUMAN STUDIES
Adjacent speech benchmarks

HOLD
CHRONIC TONSILLAR ROUTE
No direct route validation retrieved

A ESSENTIAL HEARING

Acoustic sensing → CI core + safety → Auditory output

Adjunct failures must remain inside a verified fault boundary.

B OPTIONAL ASSISTANT

PATIENT ON / OFF

Permission broker
Context + output grants

→

Model + checks
Abstain / review / confirm

→

Private output
No stimulation writes

Qualify a read-only bridge. Clinical programming uses a separate controlled path.

C NEURAL ROUTE / SEPARATE RESEARCH

No unvalidated C signal controls the hearing loop. Cuff / mesh alternatives remain unvalidated.

Sources: S1-SCL002-008; S1-ST001-014; S2 proposal pp. 25-26. Architecture is a design requirement, not a validated device.

OFF governs the adjunct. Essential battery loss or a hazardous core fault may still interrupt hearing safely.

ECONOMICS / THREE DISTINCT BOUNDARIES

Choose a boundary. Never add the values.

Conditional, unweighted project NPVs. Real 2026 USD; 15% real discount rate; 2027-2036; zero terminal value.

WHOLE PROGRAM / NPV

-$95.5m

BASE HEARING CORE

+$14.0m

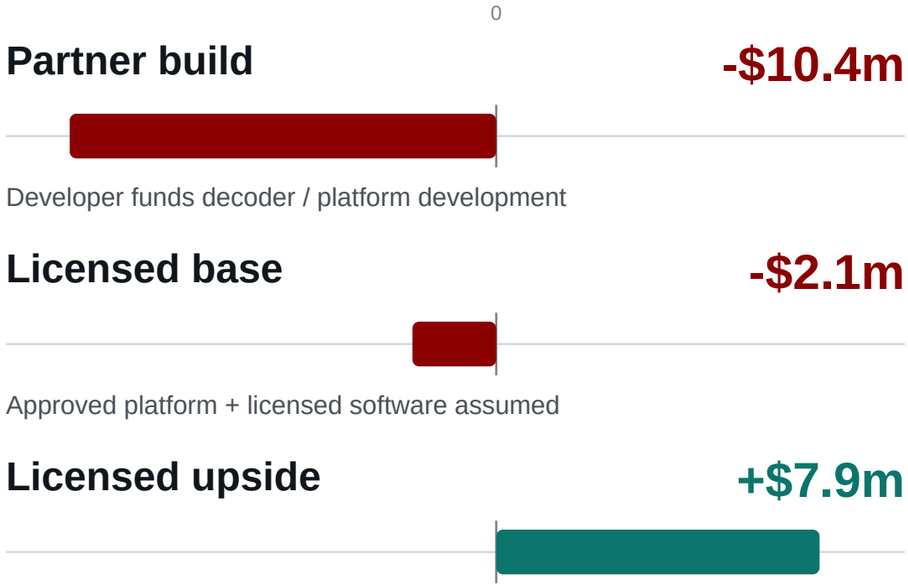
BEST + CONDITIONAL ASSISTANT

CUMULATIVE PROJECT CASH FLOW / USD M



SPONSOR HARDWARE ASSUMED / NPV

Who funds the software changes the case.



S1 alternatives: core -$85.7m; licence -$1.6m

Source: master model PPTX_Data rows 16-24, 45-62; S2 M001 / Partner option / M004. S1 alternatives remain visible.

NPVs are not equity prices. Base and best whole-program scopes differ; each boundary is a separate decision.

LICENSED PLATFORM / COMMERCIAL TEST

Price and uptake must earn the investment.

The approved-platform option is still conditional on rights, compatibility and measured patient value.

LICENSED OPTION / NPV IN USD MILLIONS

PAID ATTACH	ANNUAL FEE $600	$1,200	$1,800
30%	-8.6	-5.5	-2.7
60%	-7.7	-2.1 BASE	+2.6
90%	-6.8	+1.0	+7.9

ZERO NPV AT 60% ATTACH — $1,461 / year

ONE BILLED PATIENT-YEAR / BASE

$1,200

ANNUAL SUBSCRIPTION FEE



Service	$300
Licences	$60
Sponsor share	$120

BEFORE FIXED COSTS — **$720**

Not profit: development, tax and working capital still need funding.

Source: master PPTX_Data D90:F92 (heatmap), D103 (zero-NPV fee), D68:D71 (costs); S2 Platform option.

Original-reference sensitivity; recompute after model edits. New subscribers are billed for half a year; year-end users are not billed patient-years.

JOINT TERMS + RELEASE GATES

The next cheque follows the next proof.

Agree shared responsibilities. Stop or rescope if rights, signals, patient benefit or economics fail.

0-90 days

1

UP TO $1.5M

RIGHTS + SCOPE

- Sponsor records
- Signal-origin protocol
- Paid-value study design

Months 3-9

2

NEW APPROVAL REQUIRED

REPLICATE + COMPARE

- Credible signal or external-only scope
- Measured patient controls
- Initial comparative benefit

Months 9-18

3

NEW APPROVAL REQUIRED

PACKAGE + COST

- Authorized preclinical plan
- Reliability and manufacturing
- Service cost evidence

Months 18-24

4

NEW APPROVAL REQUIRED

INTEGRATE + FUND

- Fault tests + regulator feedback
- Executed rights
- Clinical and support obligations

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01 OWNERSHIP

- Sponsor / legal manufacturer
- Clinical and quality accountability
- Data and interface rights

02 ECONOMICS

- Contribution shares and milestones
- Licence / royalty / exclusivity
- Evidence-based commercial scope

03 CONTINUITY

- Cybersecurity and maintenance
- Recall / revision / exit obligations
- Fund surviving support cohorts

SEPARATE SUPPORT STRESS

Licensed base -$2.1m to -$7.7m | Upside +$7.9m to -$0.5m

Unfunded remaining seven-year support after 2036: fixed surviving cohorts; no renewal revenue.
Illustrative stress, not a contracted liability or base case. Fund support terms before capital release.

Sources: editorial controls; S2 proposal pp. 4-6, 22-26; S2 support run-off analysis in the companion technical review.

No chronic tonsillar human use is authorized by this investment decision. Estimates and gate dates are planning assumptions.